

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Salbutamol 100mcg MDI and Amoxicillin 500mg capsules for the treatment of COPD Management

Summary of NICE / NICE CKS Guidelines for COPD Management

COPD Overview

- **Definition:** Chronic Obstructive Pulmonary Disease is a progressive condition characterised by persistent airflow limitation due to emphysema and/or chronic bronchitis.
- **Diagnosis:** Confirmed by spirometry showing FEV1/FVC <0.70; GOLD classification (GOLD 1-4) based on FEV1.
- **Aetiology:** Predominantly smoking-related; occupational/environmental exposures possible.

GOLD Classification and Exacerbations

- **GOLD 1:** FEV1 ≥80% predicted (mild); low exacerbation risk.
- **GOLD 2:** FEV1 50-79% predicted (moderate); higher exacerbation risk.
- **GOLD 3-4:** FEV1 <50% predicted (severe/very severe); high exacerbation risk.
- **Exacerbation:** Acute worsening with increased dyspnoea, cough, and change in sputum characteristics (colour, volume).

Exacerbation Assessment

- **Infective exacerbation:** Purulent sputum (yellow/green) indicates bacterial involvement.
- **Symptoms:** Increased dyspnoea, cough, sputum production, haemoptysis, fever.
- **Severity:** Assess SpO2, RR, accessory muscle use; consider hospital referral if severe (SpO2 <88%, RR >25, inability to speak).

Pharmacological Management

- **Rescue therapy:** Short-acting beta-2 agonists for acute symptom relief (salbutamol).
- **Infective exacerbation:** Antibiotics for purulent sputum (amoxicillin for lower respiratory tract infection).
- **Oxygen therapy:** Target SpO2 88-92%; managed separately from this PGD.

Patient Group Direction

for the supply/administration of

Salbutamol 100mcg MDI inhaler

for the treatment of

COPD Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	- Pharmacist registered and practicing with the GPhC - Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. - All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines - All users must previously have used PGDs to supply medication - Must work in compliance with the SOPs of their own employer - All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	- All users must be up to date with CPD requirements and appraisal - All users must be up to date with MHRA and safety alerts with regards to medical products - All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Acute symptom relief in adults with COPD presenting with acute exacerbation or breathlessness
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of COPD (documented spirometry and GOLD classification) Acute exacerbation or episodes of breathlessness requiring symptom relief Capable of using inhaler device or willing to use spacer Capable of giving valid informed consent
Exclusion criteria	Known hypersensitivity to salbutamol or other beta-2 agonists Severe exacerbation with hypoxia (SpO₂ <88%) requiring emergency referral and oxygen therapy Acute distress with inability to speak, cyanosis, or signs of respiratory failure
Cautions	Cardiovascular disease: salbutamol can increase heart rate and blood pressure; assess cardiac risk Hypertension: monitor blood pressure; beta-2 agonists may worsen Coronary artery disease or recent MI: assess risk/benefit; monitor for

	angina Diabetes mellitus: monitor blood glucose (hyperglycaemia possible) Hyperthyroidism: beta-2 agonists can worsen symptoms Hypokalaemia: may be worsened; monitor K+ levels Pregnancy: generally safe; ensure informed consent
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Salbutamol 100mcg metered-dose inhaler (MDI)
Legal category	POM
Storage	Store below 25°C. Keep away from heat and direct sunlight. Shake well before each use.
Route/method of administration	Inhalation via MDI, with or without spacer
Dose and frequency	2-4 puffs as required for breathlessness Use for acute symptom relief during exacerbations or ongoing dyspnoea Use spacer device to optimise drug delivery if patient is not experienced with MDI May repeat at intervals as needed for symptom control
Quantity to be administered and/or supplied	1 MDI inhaler (200 doses)
Maximum or minimum treatment period	As needed (ongoing as rescue therapy)
Adverse effects	Common: fine tremor of hands, headache, nervousness, tachycardia, palpitations Uncommon: muscle cramps, anxiety, dizziness Rare: hypersensitivity reactions, arrhythmias, myocardial ischaemia, angina

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: that valid informed consent was given

	• name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each medication.
Follow-up advice to be given to patient or carer	Use the rescue inhaler (salbutamol) as needed when you experience breathlessness. Use a spacer device if you have difficulty coordinating MDI actuation. Complete the full course of amoxicillin even if symptoms improve. Take amoxicillin at regular intervals, ideally 1 hour before or 2 hours after meals for best absorption. If using oral contraception, use additional contraceptive methods during and for 7 days after the antibiotic course. Monitor your sputum colour: purulent (yellow/green) sputum suggests continued infection. Seek immediate medical attention if symptoms worsen despite treatment, or if you develop fever, persistent chest pain, or haemoptysis. Seek urgent assessment if you experience worsening breathlessness, difficulty speaking in sentences, confusion, or cyanosis. Monitor your oxygen saturation if you have a pulse oximeter at home; report any drop below 88%. Ensure you have regular follow-up with your GP to review your COPD management plan. Report any allergic reactions (rash, facial swelling, difficulty breathing) immediately.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Chronic obstructive pulmonary disease in adults
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Amoxicillin 500mg capsules

for the treatment of

COPD Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of infective acute exacerbations of COPD characterised by purulent sputum (bacterial lower respiratory tract infection)
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of COPD with spirometry evidence Acute exacerbation with purulent sputum (yellow/green coloration indicating bacterial infection) Able to take oral medication Capable of giving valid informed consent
Exclusion criteria	Known penicillin or beta-lactam allergy Infectious mononucleosis (amoxicillin can precipitate severe rash)

	Severe renal impairment (eGFR <30 mL/min/1.73m²); dose adjustment needed Antibiotic resistance suspected in local resistance patterns - check local microbiology guidance
Cautions	Mild to moderate renal impairment: monitor renal function; dose adjustment may be needed Hepatic impairment: generally safe but monitor liver function Pregnancy and breastfeeding: amoxicillin is generally safe; ensure informed consent Drug interactions: may reduce efficacy of oral contraceptives; advise additional contraception Check local antibiotic resistance patterns before prescribing (Haemophilus influenzae, Moraxella catarrhalis, Streptococcus pneumoniae coverage) Use wider-spectrum antibiotics if resistance is a concern in the area
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Amoxicillin 500mg capsules
Legal category	POM
Storage	Store below 25°C. Keep in original container. Protect from moisture.
Route/method of administration	Oral
Dose and frequency	500mg (one capsule) three times daily To be taken on an empty stomach (1 hour before or 2 hours after meals) for optimal absorption Or may be taken with food if GI upset occurs
Quantity to be administered and/or supplied	15 capsules (5-day course)
Maximum or minimum treatment period	5 days
Adverse effects	Very common: diarrhoea, nausea Common: rash (non-allergic), vomiting, abdominal pain, vaginitis Uncommon: indigestion, headache Rare: hypersensitivity reactions (urticaria, Stevens-Johnson

	syndrome, TEN), hepatitis, C. difficile infection, angioedema, anaphylaxis
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Valid from date	Expiry date		
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